NCCP 00279 · VERSION 8

Your treatment, explained

Cisplatin + etoposide with radiotherapy

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc-etoposide

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- My treatment overall
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

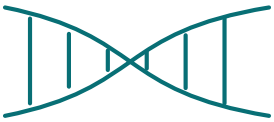
My chemotherapy medicines

Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

PLATINUM CHEMOTHERAPY

Cisplatin

Platinum chemotherapy



Damages DNA

Cisplatin forms links in DNA that interfere with cell copying. Fluids and kidney checks help reduce harm to healthy tissue.

Treatment reaches cells → Damages DNA → Cancer growth may slow

What to watch for

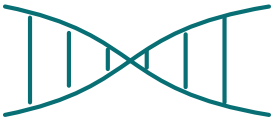
Report poor fluid intake, reduced urine, ringing in the ears or numbness.

Sources: nccp

CHEMOTHERAPY

Etoposide

Chemotherapy



Disrupts DNA repair during cell division

Etoposide blocks an enzyme cells need while copying DNA. The route and prescribed amount differ between infusion and capsules; they are not interchangeable milligram for milligram.

Treatment reaches cells → Disrupts DNA repair during cell division → Cancer growth may slow

What to watch for

Report fever, bleeding, severe mouth soreness or vomiting.

Sources: nccp · smpc-etoposide

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

My treatment overall

DAYS 1–3 EACH CYCLE

Four chemotherapy cycles



Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

SEPARATE RT APPOINTMENTS

Radiotherapy alongside chemotherapy



Usually begins with cycle 1; later start may be appropriate. The radiation team confirms dose, number of visits and timing. A chemotherapy delay does not automatically mean radiation stops.

NEXT PLAN AGREED

Recovery and response review



After treatment, your team checks recovery, persistent symptoms and response. Scan type/timing and any useful tumour-marker testing are individual decisions; this regimen does not specify a universal scan or marker timetable.

The chemotherapy course is finite; radiotherapy timing and next treatment are confirmed separately.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Required safety tests are distinct from team-agreed imaging and follow-up; no universal tumour-marker interval is specified.

Check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy	Confirm aim and prescription	Days1–3 each cycle	No additional routine doses	Repeat up to4cycles	Next treatment only if agreed
Radiotherapy	Separate planning appointment	Usually starts cycle1	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods before cycle	Follow abnormal results
Symptoms / response	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00279 v8 · official regimen

THE WHOLE TREATMENT JOURNEY

Cisplatin + etoposide with radiotherapy: treatment and checks

Limited-stage small-cell lung cancer · combined chemotherapy and radiation

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy NCCP 00279	Confirm aim and prescription	Days1–3 each cycle	No additional routine doses	Repeat up to4cycles	Next treatment only if agreed
Radiotherapy NCCP 00279	Separate planning appointment	Usually starts cycle1	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests NCCP 00279	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods before cycle	Follow abnormal results
Symptoms / response NCCP 00279	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

What each check is for

1

Prepare

Before first dose

Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

Ask / act: Confirm prescription, radiation plan and contacts.

Sources: NCCP 00279

2

Monitor

Each cycle

Check fitness and tolerability

Ask / act: Report symptoms and confirm test results before dosing.

Sources: NCCP 00279

3

Review

After course / when clinically indicated

Assess response and recovery

Ask / act: Confirm the next plan and persistent symptoms.

Sources: NCCP 00279

At the next review, ask

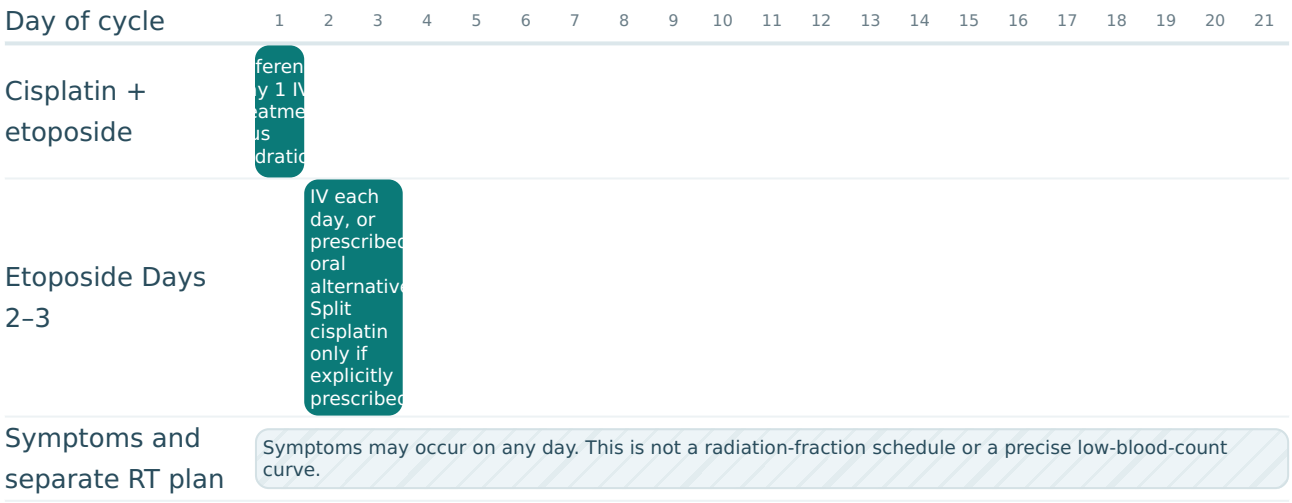
- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.



Relative schedule, not personal dates: Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

No fixed symptom day: Sickness, fatigue, low counts, swallowing problems or breathing symptoms can occur between visits and after treatment. Do not wait for a calendar window to seek advice.

Radiation is a separate schedule: Chemotherapy Day 1/Day 2/Day 3 or weekly counts are not radiation fractions. The radiation oncology team supplies the exact RT timetable.

> **Why these bands stay broad:** Reference cycle without patient dates. RT visits and dose are separately prescribed; the symptom band is not a nadir prediction. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00279 v8 · official regimen ↗

> **There are no guaranteed “safe days”.** Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty,

> chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Read the whole cycle as text

Day 1: Day 1: reference combination

Blood results, symptoms, anti-sickness medicines and hydration are checked before both drugs. Tell the nurse about any hearing, nerve or fluid-intake problems.

Day 2–3: Days 2–3: etoposide

IV treatment each day or prescribed capsules twice daily. The split-cisplatin alternative changes these visits. Use your written prescription; do not add IV and oral etoposide together.

Day 4–7: Support and recovery

No routine chemotherapy in the reference plan. Follow ongoing anti-sickness medicines and separate RT appointments. Call early for poor intake, vomiting, swallowing pain or fever.

Day 8–21: Between chemotherapy cycles

Fatigue, mouth symptoms and infection risk can persist. RT may continue according to its own plan. Report symptoms when they occur; do not wait for the next blood test.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Confirm your plan

Reference IV plan: etoposide first on Days 1–3, cisplatin after etoposide on Day 1, each infusion about one hour plus checks and fluids. If prescribed, Days 2–3 etoposide may instead be capsules twice daily. Confirm route, platinum, interval and hydration before treatment; never combine alternatives yourself.



STEP 2

Keep the contact card

The team supplies the daytime and out-of-hours numbers. Keep the card with you and show it when seeking medical care.



STEP 3

Two linked treatment teams

Confirm which team to contact for radiation or chemotherapy changes. Report swallowing problems early and ask about transport, nutrition and fatigue support.

Open practical treatment guide · **Printable practical guide**

Sources: NCCP 00279 v8 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Fluids and sickness prevention

Cisplatin needs a planned fluid and anti-sickness course. Follow the labelled medicines for each treatment day and the days afterwards. Call if vomiting prevents drinking or taking medicines.

2 If capsules are prescribed

Etoposide capsules on Days 2–3 replace the IV doses on those days. Take the prescribed amount twice daily on an empty stomach. Follow pharmacy handling instructions and contact the team if a dose is missed or vomited; do not take an extra dose without advice.

3 Radiotherapy support

Ask about swallowing, eating, skin care and fatigue during radiation. Painful swallowing or poor intake needs early help; inability to drink needs urgent assessment.

Sources: nccp · ncis

Why chemotherapy and radiotherapy are combined

Studies support combining systemic treatment with radiation in the relevant lung-cancer setting.

The evidence comes from specific treatment schedules and patient groups. Your team explains how it applies to your situation; study averages cannot predict an individual outcome.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Which drugs and routes are prescribed for me?

Confirm the exact drug names, routes and interval against your written prescription. The team must explain any change from this reference plan.

How do my radiation appointments fit around chemotherapy?

Radiation has its own calendar. Ask for the combined plan and whom to contact if a chemotherapy or radiation visit changes.

Which blood tests are required before the next dose?

Use the checks table as a prompt. Your team confirms which tests are needed, where they are done and who reviews the results before treatment.

Who do I call during the day and overnight?

Ask the team to replace the example numbers on your card with the correct daytime, overnight and backup contact routes before you leave.

When will recovery and response be reviewed?

Agree the follow-up appointments and any scans with your team. This regimen does not set a universal scan or tumour-marker timetable.

> **Try explaining the plan back:** Explain back your chemotherapy days, how radiation is scheduled separately, the tests before treatment and whom you will call if unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00279 v8. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00279 v8 · official regimen
- NCCP NCIS medical-oncology antiemetics v8
- Etoposide capsules manufacturer product information
- Turrisi et al.1999 · limited-stage SCLC radiotherapy comparison

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://assets.hse.ie/media/documents/279_CISplatin_and_Etoposide_and_RT.pdf

https://assets.hse.ie/media/documents/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://www.medicines.org.uk/emc/product/10533/smpc

https://www.nejm.org/doi/full/10.1056/NEJM199901283400403